

A - ADMINISTRATIVE INFORMATION - PATIENT TRACKING

(M0010) C M S Certification Number:

(M0014) Branch State:

(M0016) Branch ID Number:

(M0018) National Provider Identifier (N P I) for the attending physician who has signed the plan of care:

☐ UK - Unknown or Not Available

(M0020) Patient ID Number:

(M0030) Start of Care Date:

(M0040) Patient Name:

(M0050) Patient State of Residence:

(M0060) Patient ZIP Code:

(M0063) Medicare Number: ☐ NA - No Medicare

(M0064) Social Security Number: ☐ UK - Unknown or Not Available

(M0065) Medicaid Number: ☐ NA - No Medicaid

(M0066) Birth Date:

(M0069) Gender:

(A1005) Ethnicity: (Check all that apply.)

Are you of Hispanic, Latino/a, or Spanish origin?

☐ A. No, not of Hispanic, Latino/a, or Spanish origin

☐ B. Yes, Mexican, Mexican American, Chicano/a

☐ C. Yes, Puerto Rican

☐ D. Yes, Cuban

☐ E. Yes, another Hispanic, Latino, or Spanish origin

☐ X. Patient unable to respond

☐ Y. Patient declines to respond

(A1010) Race (Check all that apply.)

What is your race?

☐ A. White

☐ B. Black or African American

☐ C. American Indian or Alaska Native

☐ D. Asian Indian

☐ E. Chinese

☐ F. Filipino

☐ G. Japanese

☐ H. Korean

☐ I. Vietnamese

☐ J. Other Asian

☐ K. Native Hawaiian

☐ L. Guamanian or Chamorro

☐ M. Samoan

☐ N. Other Pacific Islander

☐ X. Patient unable to respond

☐ Y. Patient declines to respond

☐ Z. None of the above

(M0150) Current Payment Sources for Home Care: (Mark all that apply.)

☐ 0 - None; no charge for current services

☐ 1 - Medicare (traditional fee-for-service)

☐ 2 - Medicare (HMO/managed care/Advantage plan)

☐ 3 - Medicaid (traditional fee-for-service)

☐ 4 - Medicaid (HMO/managed care)

☐ 5 - Workers' Compensation

☐ 6 - Title programs (for example, Title III, V, or XX)

☐ 7 - Other government (for example, TriCare, VA.)

☐ 8 - Private Insurance

☐ 9 - Private HMO/managed care

☐ 10 - Self-pay

☐ 11 - Other (specify)

☐ UK - Unknown

A - Administrative Information (Clinical)

(A1110) Language

A. What is your preferred language?

☐ No Information

B. Do you need or want an interpreter to communicate with a doctor or health care staff?

☐ No ☐ Yes ☐ Unable to determine

(M0080) Discipline of Person Completing Assessment:

☐ 1-RN ☐ 2-PT ☐ 3-SLP/ST ☐ 4-OT

(M0090) Date Assessment Completed:

(M0100) This Assessment is Currently Being Completed for the Following Reason:

Start/Resumption of Care

☒ 1 - Start of care-further visits planned

☐ 3 - Resumption of care (after inpatient stay)

Follow-Up

☐ 4 - Recertification (follow-up) reassessment

☐ 5 - Other follow-up

Transfer to an Inpatient Facility

☐ 6 - Transferred to an inpatient facility-patient not discharged from agency

☐ 7 - Transferred to an inpatient facility-patient discharged from agency

Discharge from Agency - Not to an Inpatient Facility

☐ 8 - Death at home

☐ 9 - Discharge from agency

(M0102) Date of Physician-ordered Start of Care (Resumption of Care): If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.

☐ NA - No specific SOC date ordered by physician

(M0104) Date of Referral: Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA.

(A1250) Transportation: (NACHC @) (Check all that apply.)

Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

☐ A. Yes, it has kept me from medical appointments or from getting my medications

☐ B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need

☐ C. No

☐ X. Patient unable to respond

☐ Y. Patient declines to respond

(M1000) From which of the following Inpatient Facilities was the patient discharged within the past 14 days? (Mark all that apply.)

☐ 1 - Long-term nursing facility (NF)

☐ 2 - Skilled nursing facility (SNF / TCU)

☐ 3 - Short-stay acute hospital (IPP S)

☐ 4 - Long-term care hospital (LTCH)

☐ 5 - Inpatient rehabilitation hospital or unit (IRF)

☐ 6 - Psychiatric hospital or unit

☐ 7 - Other (specify)

☐ NA - Patient was not discharged from an inpatient facility

(M1005) Inpatient Discharge Date (most recent):

☐ UK - Unknown or not available

Pick 2:
N/A:
Comments:

Temperature:
Temperature Type:
Pulse:
Pulse Type:
Respirations:
O2 saturation:
Oxygen:
Frequency of Oxygen:
Oxygen L/min:
Systolic Blood Pressure Sitting:
Diastolic Blood Pressure Sitting:
Systolic Blood Pressure Lying:
Diastolic Blood Pressure Lying:
Systolic Blood Pressure Standing:
Diastolic Blood Pressure Standing:
Location for BP:
Height:
Weight:
Weight is:

Does patient have complaints of pain? (Chronic or
present day acute):

**NUMERIC RATING SCALE (NRS) 0=no
pain/10=worst pain possible**

Pain Scale:
Pain Location:
Radiation of pain:
Date of Onset:
Pain Description:
Non-verbals demonstrated:
Reason for non-verbal assessment:
Pain pattern:
Pain frequency:
Factors such as activities, care, or treatment that
precipitate or exacerbate pain:
What makes the pain better:
What makes the pain worse:
Time of day/night when pain is worse:
Does pain impact function?:
Does pain impact quality of life:
How:
Current Pain Med. Regimen (med/dose/freq/route):
Nonpharmacological pain control :

Time of last pain medication taken:
Total use in the last 24 hours:
Pain medication utilized appropriately:
Pain log utilized:
Is current pain control regimen effective:
Patient/Family's goals for pain management:
Patient/family's satisfaction with the current level of
pain control:
Notified Physician:
Pain Assessment Documentation/Teaching:

***** Loc - Advanced Directives *****

Advanced Directives:
Does Agency Have a Copy of the Advanced
Directives:

B - HEARING, SPEECH AND VISION

(B0200) Hearing. Ability to hear (with hearing aid or hearing appliances if normally used).

- ☐ 0 - Adequate - no difficulty in normal conversation, social interaction, listening to TV
- ☐ 1 - Minimal difficulty - difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- ☐ 2 - Moderate difficulty - speaker has to increase volume and speak distinctly
- ☐ 3 - Highly impaired - absence of useful hearing
- ☐ 99 - No Information

(B1000) Vision. Ability to see in adequate light (with glasses or other visual appliances).

- ☐ 0 - Adequate - sees fine detail, such as regular print in newspapers/books
- ☐ 1 - Impaired - sees large print, but not regular print in newspapers/books
- ☐ 2 - Moderately impaired - limited vision; not able to see newspaper headlines but can identify objects
- ☐ 3 - Highly impaired - object identification in question, but eyes appear to follow objects
- ☐ 4 - Severely impaired - no vision or sees only light, colors or shapes; eyes do not appear to follow objects
- ☐ 99 - No Information

(B1300) Health Literacy (From Creative Commons ©). How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ☐ 0 - Never
- ☐ 1 - Rarely
- ☐ 2 - Sometimes
- ☐ 3 - Often
- ☐ 4 - Always
- ☐ 7 - Patient declines to respond
- ☐ 8 - Patient unable to respond

C - COGNITIVE PATTERNS

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients.

- ☐ 0 - No (patient is rarely/never understood) → Skip to C1310, Signs and Symptoms of Delirium (from CAM ©)

- ☐ 1 - Yes → Continue to C0200, Repetition of Three Words
- ☐ 99 - No Information

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of 3 words.

Ask patient: "I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: sock, blue, and bed. Now tell me the three words."

Number of words repeated after first attempt:

- ☐ 0 - None
- ☐ 1 - One
- ☐ 2 - Two
- ☐ 3 - Three
- ☐ 99 - No Information

After the patient's first attempt, repeat the words using cues ("sock, something to wear; blue, a color; bed, a piece of furniture"). You may repeat the words up to two more times.

(C0300) Temporal Orientation.

(Orientation to year, month, and day)

Ask patient: "Please tell me what year it is right now."

A. Able to report correct year

- ☐ 0 - Missed by > 5 years or no answer
- ☐ 1 - Missed by 2-5 years
- ☐ 2 - Missed by 1 year
- ☐ 3 - Correct
- ☐ 99 - No Information

Ask patient: "What month are we in right now?"

B. Able to report correct month

- ☐ 0 - Missed by > 1 month or no answer
- ☐ 1 - Missed by 6 days to 1 month
- ☐ 2 - Accurate within 5 days
- ☐ 99 - No Information

Ask patient: "What day of the week is today?"

C. Able to report correct day of the week

- ☐ 0 - Incorrect or no answer
- ☐ 1 - Correct
- ☐ 99 - No Information

(C0400) Recall.

Ask patient: "Let's go back to an earlier question. What were those three words that I asked you to repeat?"

If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word.

A. Able to recall "sock"

- ☐ 0 - No - could not recall
- ☐ 1 - Yes, after cueing ('something to wear')
- ☐ 2 - Yes, no cue required
- ☐ 99 - No Information

B. Able to recall "blue"

- ☐ 0 - No - could not recall
- ☐ 1 - Yes, after cueing ('a color')
- ☐ 2 - Yes, no cue required
- ☐ 99 - No Information

C. Able to recall "bed"

- ☐ 0 - No - could not recall
- ☐ 1 - Yes, after cueing ('a piece of furniture')
- ☐ 2 - Yes, no cue required
- ☐ 99 - No Information

(C0500) BIMS Summary score

Add scores for questions C0200-C0400 and fill in total score (00-15)
Enter 99 if the patient was unable to complete the interview

(C1310) Signs and Symptoms of Delirium (from CAM ©)

Code after completing Brief Interview for Mental Status and reviewing medical record.

A. Acute Onset of Mental Status Change - Is there evidence of an acute change in mental status from the patient's baseline?

- ☐ 0 - No
- ☐ 1 - Yes
- ☐ 99 - No Information

B. Inattention - Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ☐ 0 - Behavior not present
- ☐ 1 - Behavior continuously present, does not fluctuate
- ☐ 2 - Behavior present, fluctuates (comes and goes, changes in severity)
- ☐ 99 - No Information

C. Disorganized thinking - Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ☐ 0 - Behavior not present
- ☐ 1 - Behavior continuously present, does not fluctuate
- ☐ 2 - Behavior present, fluctuates (comes and goes, changes in severity)
- ☐ 99 - No Information

D. Altered level of consciousness - Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ** vigilant - startled easily to any sound or touch
- ** lethargic - repeatedly dozed off when being asked questions, but responded to voice or touch
- ** stuporous - very difficult to arouse and keep aroused for the interview
- ** comatose - could not be aroused

- ☐ 0 - Behavior not present
- ☐ 1 - Behavior continuously present, does not fluctuate
- ☐ 2 - Behavior present, fluctuates (comes and goes, changes in severity)
- ☐ 99 - No Information

(M1700) Cognitive Functioning: Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands.

- ☐ 0 - Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- ☐ 1 - Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions
- ☐ 2 - Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention), or consistently requires low stimulus environment due to distractibility.
- ☐ 3 - Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- ☐ 4 - Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- ☐ 0 - Never
- ☐ 1 - In new or complex situations only
- ☐ 2 - On awakening or at night only
- ☐ 3 - During the day and evening, but not constantly
- ☐ 4 - Constantly
- ☐ NA - Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- ☐ 0 - None of the time
- ☐ 1 - Less often than daily
- ☐ 2 - Daily, but not constantly
- ☐ 3 - All of the time
- ☐ NA - Patient nonresponsive

EYES

WNL:
 Visual Aids:
 Visual Impairment Left:
 Visual Impairment Right:
 Other Visual:

EARS

WNL:
 Hearing Aids:
 Hearing Impairment Left:
 Hearing Impairment Right:
 Other Hearing:

ORAL/NASAL/THROAT

WNL:
 Dentition:
 Oral Hygiene:
 Lips/Tongue:
 Nose/Throat:
 Other Oral/Nasal/Throat:
 Above Diagnoses Confirmed With Physician:
 Sensory- Additional and/or Supporting
 Documentation/Education:

D - MOOD

(D0150) Patient Mood Interview (PHQ-2 to 9)

Determine if the patient is rarely/never understood verbally, in writing, or using another method.
 If rarely/never understood, code D0150A1 and D0150B1 as 9, no response, leave D0150A2 and D0150B2 blank,
 end the PHQ-2 interview and leave D0160, Total Severity Score blank.
 Otherwise, say to patient: "Over the last 2 weeks, have you been bothered by any of the following problems?"
 If symptom is present, enter 1 (yes) in column 1, Symptom Presence.
 If yes in column 1, then ask the patient: "About how often have you been bothered by this?"
 Read and show the patient a card with the symptom frequency choices. Indicate response in column 2,
 Symptom Frequency.
 Response values for A thru I.

A. Little interest or pleasure in doing things

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

B. Feeling down, depressed, or hopeless

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

If either D0150A2 or D0150B2 is coded 2 or 3, CONTINUE asking the questions below. If not, END the PHQ interview.

C. Trouble falling or staying asleep, or sleeping too much

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

D. Feeling tired or having little energy

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

E. Poor appetite or overeating

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

F. Feeling bad about yourself - or that you are a failure or have let yourself or your family down

1. Symptom Presence

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

G. Trouble concentrating on things, such as reading the newspaper or watching television**1. Symptom Presence**

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

H. Moving or speaking so slowly that other people could have noticed. Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual**1. Symptom Presence**

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

I. Thoughts that you would be better off dead, or of hurting yourself in some way**1. Symptom Presence**

- ☐ 0. No (enter 0 in column 2)
- ☐ 1. Yes (enter 0-3 in column 2)
- ☐ 9. No response (leave column 2 blank)
- ☐ No Information

2. Symptom Frequency

- ☐ 0. Never or 1 day
- ☐ 1. 2-6 days (several days)
- ☐ 2. 7-11 days (half or more of the days)
- ☐ 3. 12-14 days (nearly every day)
- ☐ No Information

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(D0160) Total Severity Score

Add scores for all frequency responses in Column 2, Symptom Frequency.

Total score must be between 00 and 27.

Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)

(D0700) Social Isolation

How often do you feel lonely or isolated from those around you?

- ☐ 0 - Never
- ☐ 1 - Rarely
- ☐ 2 - Sometimes
- ☐ 3 - Often
- ☐ 4 - Always

○ 7 - Patient declines to respond

○ 8 - Patient unable to respond

Loc 19 - Mental Status

Mental Status:

Other:

Describe:

Neuro/Psychosocial/Financial Assessment

NEURO ASSESSMENT

WNL:

PERRLA:

Orientation:

Motor/Gait:

Sensation:

Strength:

Speech:

Communication strengths and barriers and language
skills:

Ask patient - What worries you most when you get
confused or can't remember? Enter response:

Other Neuro :

Specify if recent:

Sensory loss:

Specify:

Paralysis:

Other paralysis:

Neuro - Documentation/Teaching:

PSYCHOSOCIAL/FINANCIAL ASSESSMENT

WNL:

Functional/Social:

Dependencies:

Self Esteem:

Financial/Transportation:

Spiritual:

Religion/Clergy name and number:

Primary Language if not English:

Cultural issues impacting care :

Report to APS/CPS:

Psychosocial/Financial - Documentation/Teaching:

CCSDD: Cornell Scale for Depression in Dementia (On Demand)

Ratings should be based on symptoms and signs occurring the week before interview. No score should be given if symptoms result from physical disability or illness. Scoring: a (0) = Unable to evaluate, 0 = Absent, 1 = Mild to Intermittent, 2 = Severe Add total of all entries; sum is equal to total score.

Mood-Related Signs

1. Anxiety; anxious expression, rumination, worrying:
2. Sadness; sad expression, sad voice, tearfulness:
3. Lack of reaction to pleasant events:
4. Irritability; annoyed, short tempered:

Behavioral Disturbance

5. Agitation; restlessness, hand wringing, hair pulling:
6. Retardation; slow movements, slow speech, slow reactions:
7. Multiple physical complaints (score 0 if gastrointestinal symptoms only):
8. Loss of interest; less involved in usual activities (score 0 only if change occurred acutely, i.e., in less than one month):

Physical Signs

9. Appetite loss; eating less than usual:
10. Weight loss (score 2 if greater than 5 pounds in one month):
11. Lack of energy; fatigues easily, unable to sustain activities:

Cyclic Functions

12. Diurnal variation of mood; symptoms worse in the morning:
13. Difficulty falling asleep; later than usual for this individual:
14. Multiple awakenings during sleep:
15. Early morning awakening; earlier than usual for this individual:

Ideational Disturbance

16. Suicidal; feels life is not worth living:
17. Poor self-esteem; self-blame, self-depreciation, feelings of failure:
18. Pessimism; anticipation of the worst:
19. Mood congruent delusions; delusions of poverty, illness or loss:

SCORE:

E - Behavior

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.)

- ☐ 1 - Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- ☐ 2 - Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- ☐ 3 - Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- ☐ 4 - Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- ☐ 5 - Disruptive, infantile, or socially inappropriate behavior (**excludes** verbal actions)
- ☐ 6 - Delusional, hallucinatory, or paranoid behavior
- ☐ 7 - None of the above behaviors demonstrated

☐ 0 - Never

☐ 1 - Less than once a month

☐ 2 - Once a month

☐ 3 - Several times each month

☐ 4 - Several times a week

☐ 5 - At least daily

*Respiratory Assessment WNL?:

Shortness of Breath/Dyspnea:

Comment:

Lung Sounds

LUL:

LLL:

RUL:

RML:

RLL:

Cough:

Sputum Amount:

Hemoptysis:

Frequency:

Color :

Secretions:

Abnormal Breathing Pattern:

Oxygen Use :

Liters/per min:

Sleep apnea:

Tracheostomy/Management:

Maintained by:

Tracheostomy size:

Suction:

Suction Amount:

Ventilator:

Tobacco Use:

Respiratory -Documentation/Teaching:

Cardiac Assessment

CARDIAC HISTORY

Does Patient have a Cardiac History?:

CHF:

Other:

Arrhythmia:

CAD:

Hypertension:

Above Dx Confirmed with Physician:

ASSESSMENT

WNL:

Heart Sounds:

Apical Pulse:

Right Radial Pulse:

Left Radial Pulse:

Right Pedal Pulse:

Left Pedal Pulse:

Rhythm:

Skin Color:

Chest Pain:

Pacemaker:

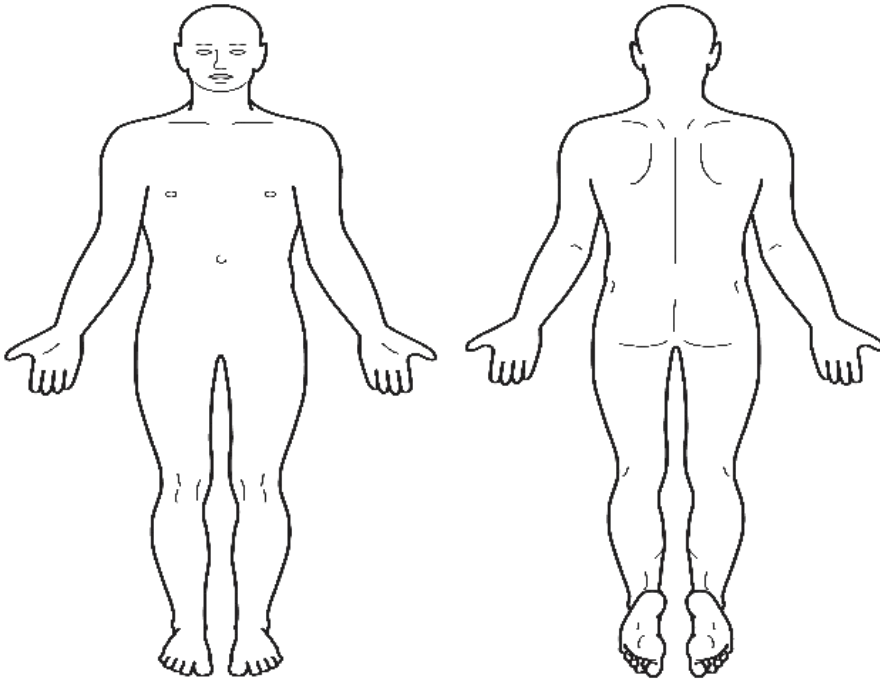
Date inserted:

Rate:
 AICD:
 Date Inserted:
 Capillary Refill:
 Other Abnormalities:
 Edema (left):
 Location/Describe:
 Edema (right):
 Location/Describe:
 Cardiovascular-Documentation/Teaching:

F - Preferences for Customary Routine Activities (Care Management)

(M2102) Types and Sources of Assistance: Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff. (Check only **one** box in each row.)

Type of Assistance	No assistance needed - patient is independent or does not have needs in this area	Non-agency caregiver(s) currently provide assistance	Non-agency caregiver(s) need training/supportive services to provide assistance	Non-agency caregiver(s) <u>not likely</u> to provide assistance OR it is <u>unclear</u> if they will provide assistance	Assistance needed, but no non-agency caregiver(s) available
f. Supervision and safety (for example, due to cognitive impairment)	☐ 0	☐ 1	☐ 2	☐ 3	☐ 4



WNL:

Skin Turgor:

Skin Color:

Skin Condition:

Skin Alterations:

Location:

Skin Integrity:

Does patient have any wounds?:

Integumentary-Documentation/Teaching:

Negative Pressure Wound Therapy

Patient pre-medicated for pain?:

Supplies:

(list):

Location:

Wound Measurement: Length (cm)

Wound Measurement: Width (cm)

Wound Measurement: Depth (cm)

Tunneling Present?:

At (O'clock):

Measurement in cm:

Undermining Present?:

At (O'clock):

Measurement in cm:

Wound Bed Description:

Eschar (necrotic tissue) description:

Necrosis %:

Slough %:

Granulation %:

Epithelial %:

Wound Margins Description:

Maceration:

Location:

Erythema:

Location:

Exudate Amount:

Exudate Consistency:

Odor:

Exudate Color:

Wound VAC Dressing Change

Old Dressing Removed, Procedure:

Wound Cleansed with:

(describe):

Wound Edges Prepped:

(describe):

Window Paning done?:

Foam Used, type:

Number of Pieces Used:

Additional Comments:

Did patient experience pain with dressing change?:

Wound Vac Settings:

Air Leak Present?:

Vacuum Cannister Replaced:

Next Dressing Change Due:

PleurX Drainage System

PleurX Drainage System in Place for:

Location of Site:

Catheter site appearance:

Patency:

Amount of drainage (cc):

Color of drainage:

Character of drainage:

Site Care Provided:

Patient's reaction to procedure:

Patient/Family Education Provided:

Physician Notification Required?:

Physician Notified:

Time/Date of Notification:

Orders Received:

If yes, details:

Wound - Burns 1

Healing Status :

:-

Location:

Extremity:

Above Diagnoses Confirmed with physician:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
First Degree:
Second Degree:
Third Degree:

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

***** Wound - Burns 2 *****

Healing Status :
-:
Location:
Extremity:
Above Diagnoses Confirmed with physician:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
First Degree:
Second Degree:
Third Degree:

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:

Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

***** Wound - Diabetic Ulcer 1 *****

Healing Status :
-:

Co-Morbidities:
Above Diagnoses Confirmed with physician:
Location:
Extremity:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:
Fat Layer Exposed:
Muscle Exposed:
Bone Exposed:
Other Severity Not Classified Above:
Description of severity :

Unspecified (risk for billing audit if used):

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:

Spoke With:

Wound - Diabetic Ulcer 2

Healing Status :

-:

Co-Morbidities:

Above Diagnoses Confirmed with physician:

Location:

Extremity:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:

Fat Layer Exposed:

Muscle Exposed:

Bone Exposed:

Other Severity Not Classified Above:

Description of severity :

Unspecified (risk for billing audit if used):

Drainage:

Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:

Odor:

Wound Cleansed With:

Topicals Used:

Other:

Dressings Applied:

Other:

Dressing Secured With:

Other:

Non-Removable Dressing:

Other [Will Need MD Order]:

Pain:

MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

Wound - Diabetic Ulcer 3

Healing Status :

-:

Co-Morbidities:

Above Diagnoses Confirmed with physician:

Location:
Extremity:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:
Fat Layer Exposed:
Muscle Exposed:
Bone Exposed:
Other Severity Not Classified Above:
Description of severity :

Unspecified (risk for billing audit if used):

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

*****	Wound - Other Wound 1	*****
-------	------------------------------	-------

Healing Status :
-:
Location:
Extremity - (Only one extremity per wound):
Type of Wound:
Confirmed with Physician:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
Wound Bed:
Tissue:

Sutures/Staples Present:
Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

Wound - Other Wound 2

Healing Status :
-:
Location:
Extremity - (Only one extremity per wound):
Type of Wound:
Confirmed with Physician:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
Wound Bed:
Tissue:
Sutures/Staples Present:
Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

Wound - Other Wound 3

Healing Status :

-:

Location:

Extremity - (Only one extremity per wound):

Type of Wound:

Confirmed with Physician:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

Wound Bed:

Tissue:

Sutures/Staples Present:

Drainage:

Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:

Odor:

Wound Cleansed With:

Topicals Used:

Other:

Dressings Applied:

Other:

Dressing Secured With:

Other:

Non-Removable Dressing:

Other [Will Need MD Order]:

Pain:

MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

Wound - Pressure Ulcer 1

Healing Status :

-:

Confirmed with physician:

Location:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

Stage 1:
Stage 2:
Stage 3:
Stage 4:
Unstageable:
Closed:
Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

*****	Wound - Pressure Ulcer 2	*****
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Healing Status :
-:
Confirmed with physician:
Location:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
Stage 1:
Stage 2:
Stage 3:
Stage 4:
Unstageable:
Closed:
Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:

Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

***** Wound - Pressure Ulcer 3 *****

Healing Status :
-:
Confirmed with physician:
Location:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
Stage 1:
Stage 2:
Stage 3:
Stage 4:
Unstageable:
Closed:
Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

***** Wound - Surgical Wound 1 *****

Healing Status :
-:

See Surgical Wounds Documented In Other Sections:

Location:

Reason for Surgical Wound (Diagnoses):

Reason for Surgical Wound Confirmed With
Physician:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

Newly Epithelialized:

Fully Granulating:

Early/Partial Granulation:

Not Healing:

Sutures/Staples Present:

Number Sutures/Staples Present:

Drainage:

Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:

Odor:

Wound Cleansed With:

Topicals Used:

Other:

Dressings Applied:

Other:

Dressing Secured With:

Other:

Non-Removable Dressing:

Other [Will Need MD Order]:

Pain:

MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

Wound - Surgical Wound 2

Healing Status :

-:

See Surgical Wounds Documented In Other Sections:

Location:

Reason for Surgical Wound (Diagnoses):

Reason for Surgical Wound Confirmed With
Physician:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

Newly Epithelialized:

Fully Granulating:

Early/Partial Granulation:
Not Healing:
Sutures/Staples Present:
Number Sutures/Staples Present:

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

***** **Wound - Surgical Wound 3** *****

Healing Status :
-:

See Surgical Wounds Documented In Other Sections:

Location:
Reason for Surgical Wound (Diagnoses):
Reason for Surgical Wound Confirmed With
Physician:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:
Newly Epithelialized:
Fully Granulating:
Early/Partial Granulation:
Not Healing:
Sutures/Staples Present:
Number Sutures/Staples Present:

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:

Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

*****	Wound - Surgical Wound 4	*****
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Healing Status :

-:

See Surgical Wounds Documented In Other Sections:

Location:
Reason for Surgical Wound (Diagnoses):
Reason for Surgical Wound Confirmed With
Physician:

Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

Newly Epithelialized:
Fully Granulating:
Early/Partial Granulation:
Not Healing:
Sutures/Staples Present:
Number Sutures/Staples Present:

Drainage:
Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:

Spoke With:

Wound - Venous Ulcer 1

Healing Status :

-:

Co-Morbidities:

Above Diagnoses Confirmed with physician:

Location:

Extremity:

Length (cm) [head to toe] :

Width (cm) [side to side] :

Depth (cm) :

Undermining (cm) [enter zero for none]:

Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:

Fat Layer Exposed:

Muscle Exposed:

Bone Exposed:

Other Severity Not Classified Above:

Description of severity :

Unspecified (risk for billing audit if used):

Drainage:

Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:

Odor:

Wound Cleansed With:

Topicals Used:

Other:

Dressings Applied:

Other:

Dressing Secured With:

Other:

Non-Removable Dressing:

Other [Will Need MD Order]:

Pain:

MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

Wound - Venous Ulcer 2

Healing Status :

-:

Co-Morbidities:

Above Diagnoses Confirmed with physician:

Location:
Extremity:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:
Fat Layer Exposed:
Muscle Exposed:
Bone Exposed:
Other Severity Not Classified Above:
Description of severity :

Unspecified (risk for billing audit if used):

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

*****	Wound - Venous Ulcer 3	*****
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Healing Status :
-:
Co-Morbidities:
Above Diagnoses Confirmed with physician:
Location:
Extremity:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:

Fat Layer Exposed:
Muscle Exposed:
Bone Exposed:
Other Severity Not Classified Above:
Description of severity :

Unspecified (risk for billing audit if used):

Drainage:
Drainage Type [if drainage, must choose 1]:
Surrounding Tissue:
Odor:
Wound Cleansed With:
Topicals Used:
Other:
Dressings Applied:
Other:
Dressing Secured With:
Other:
Non-Removable Dressing:
Other [Will Need MD Order]:

Pain:
MD Notified Of :
Spoke With:

RN Notified:
Spoke With:

*****	Wound - Venous Ulcer 4	*****
-------	-------------------------------	-------

Healing Status :
-:
Co-Morbidities:
Above Diagnoses Confirmed with physician:
Location:
Extremity:
Length (cm) [head to toe] :
Width (cm) [side to side] :
Depth (cm) :
Undermining (cm) [enter zero for none]:
Tunneling (cm) [enter zero for none]:

STRUCTURES EXPOSED

Breakdown of Skin:
Fat Layer Exposed:
Muscle Exposed:
Bone Exposed:
Other Severity Not Classified Above:
Description of severity :

Unspecified (risk for billing audit if used):

Drainage:

Drainage Type [if drainage, must choose 1]:

Surrounding Tissue:

Odor:

Wound Cleansed With:

Topicals Used:

Other:

Dressings Applied:

Other:

Dressing Secured With:

Other:

Non-Removable Dressing:

Other [Will Need MD Order]:

Pain:

MD Notified Of :

Spoke With:

RN Notified:

Spoke With:

*****	Wound - Education	*****
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Taught s/sx to report to SN/MD:

Understanding voiced:

WC Taught To:

Return demonstration:

WC Narrative:

(M1800) Grooming: Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, fingernail care).

- ☐ 0 - Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- ☐ 1 - Grooming utensils must be placed within reach before able to complete grooming activities.
- ☐ 2 - Someone must assist the patient to groom self.
- ☐ 3 - Patient depends entirely upon someone else for grooming needs.

(M1810) Current Ability to Dress Upper Body safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- ☐ 0 - Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- ☐ 1 - Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- ☐ 2 - Someone must help the patient put on upper body clothing.
- ☐ 3 - Patient depends entirely upon another person to dress the upper body.

(M1820) Current Ability to Dress Lower Body safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- ☐ 0 - Able to obtain, put on, and remove clothing and shoes without assistance.
- ☐ 1 - Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- ☐ 2 - Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- ☐ 3 - Patient depends entirely upon another person to dress lower body.

(M1830) Bathing: Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- ☐ 0 - Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- ☐ 1 - With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- ☐ 2 - Able to bathe in shower or tub with the intermittent assistance of another person:
 - (a) for intermittent supervision or encouragement or reminders, OR
 - (b) to get in and out of the shower or tub, OR
 - (c) for washing difficult to reach areas.
- ☐ 3 - Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ☐ 4 - Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ☐ 5 - Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person.
- ☐ 6 - Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring: Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode.

- ☐ 0 - Able to get to and from the toilet and transfer independently with or without a device.
- ☐ 1 - When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ☐ 2 - Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ☐ 3 - Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently.
- ☐ 4 - Is totally dependent in toileting.

(M1845) Toileting Hygiene: Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ☐ 0 - Able to manage toileting hygiene and clothing management without assistance.
- ☐ 1 - Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ☐ 2 - Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ☐ 3 - Patient depends entirely upon another person to maintain toileting hygiene.

Comments

(M1850) Transferring: Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast.

- ☐ 0 - Able to independently transfer.
- ☐ 1 - Able to transfer with minimal human assistance or with use of an assistive device.
- ☐ 2 - Able to bear weight and pivot during the transfer process but unable to transfer self.
- ☐ 3 - Unable to transfer self and is unable to bear weight or pivot when transferred by another person.
- ☐ 4 - Bedfast, unable to transfer but is able to turn and position self in bed.
- ☐ 5 - Bedfast, unable to transfer and is unable to turn and position self.

Comments

- (M1860) Ambulation/Locomotion:** Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.
- ☐ 0 - Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
 - ☐ 1 - With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
 - ☐ 2 - Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
 - ☐ 3 - Able to walk only with the supervision or assistance of another person at all times.
 - ☐ 4 - Chairfast, unable to ambulate but is able to wheel self independently.
 - ☐ 5 - Chairfast, unable to ambulate and is unable to wheel self.
 - ☐ 6 - Bedfast, unable to ambulate or be up in a chair.

Comments

*****	Nutritional Assessment	*****
-------	-------------------------------	-------

Diet:
 Favorite Foods:
 Enteral Feeding:
 Enteral Nutrition:
 Per:
 Who manages:
 Appearance of Insertion Site:
 Type of Feeding/Amount/Rate:
 Flushing Protocol (amount/how often):
 Feeding Bag Change/Time:
 Oral Liquids via:
 Hydration Status:

Nutritional Screen

SOC, ROC, Recert Visit OR pt change in condition
 and requires Nutrition Screen?:
 Complex wounds (3):
 TPN therapy (3):
 Impaired/inadequate food intake (2):
 Eats less than 2 meals a day (3):
 Eats few fruits, vegetables or milk products (2):
 Tooth, mouth or swallowing problems (2):
 Insufficient money to buy food (4):
 Eats alone (1):
 Takes 3 or more meds (1):
 Invol. wgt. loss/gain of 10 lbs in past 6 mos (2) :
 Total :

0-5 = Low Nutritional Risk

**(Continue to observe for nutritional needs and
 intervene as necessary)**

6-9 - Moderate Risk

**(Educate the Patient/family/caregiver to improve
 eating habits & lifestyle including consideration
 for pt's food preference and frequency of meals.**

Involve the R.D. as needed for educational

materials or suggestions in improvement
measures)

10+ = High nutritional risk

(RN to consult with R.D. consult w/the physician,
consider labs, wgt changes, diet. Send written
communication to the R.D. Obtain order for R.D.
as needed)

Nutrition:
Appetite:
Meal Pattern:
Swallowing Difficulty (describe):
Education:
Who was taught:
How taught:
Nutrition - Additional Information:

Loc 16 - Nutritional Requirements

Nutritional Requirements:

Other:

Endocrine/Hematopoietic Assessment v2

WNL:
Endocrine Disorders (excluding diabetes):

Other:

Endocrine Symptoms/Manifestations:

Other detail:

Diabetes/Blood Glucose Monitoring

Is diabetic or requires blood glucose
monitoring/intervention for other condition:

Type of Diabetes:

Diabetes History:

Approximate date of onset/diagnosis:

Manifestations of Complications :

Management:

All Above Diagnoses Confirmed With Physician?:

FBS Today:

RBS Today:

Recent symptoms of Hyper/Hypoglycemia?:

Diabetic Foot Exam (performed by):

Results of Foot Exam:

Glucose Monitoring

Patient:
Glucose Meter:
Checked By:
Range Since Last SNV:
Reported Range within physician ordered
parameters?:

Frequency Checked:

Insulin Administration

Insulin Injection or Insulin Pump Administration:

Pt/CG capable of drawing up/measuring/administering
insulin:

Insulin drawn up/measured/administered by:

Insulin Pump

Rate/volume infused/volume filled:

If applicable, Insulin pump filled and changed by:

Insulin pump frequency of change:

Location placed:

Next change due:

Education Provided/Provided to/How provided:

Hematopoietic Assessment

Hematology WNL?:

Hematological Problems:

(specify):

Additional Information/Comments/Education:

*****	Diabetic Foot Exam	*****
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Has there been a change in the foot in the last month?:

Comments (1):

Is there a foot ulcer now or history of one?:

Comments (2):

Is there pain in the calf muscle when walking or at
rest?:

Comments (3):

Does the foot have an abnormal shape?:

Comments (4):

Is there weakness in the ankle or foot?:

Comments (5):

Are the nails thick, too long or ingrown?:

Comments (6):

R Posterior Tibial Pulse:

L Posterior Tibial Pulse:

R Dorsalis Pedis:

L Dorsalis Pedis:

Sensory Foot Exam

**Label SENSORY level with a "+" in the areas of
the foot if patient can feel the monofilament**

**Label SENSORY level with a "-" in the areas of
the foot if patient cannot feel the monofilament**

R anterior mid-foot:

L anterior mid-foot:

R plantar distal mid-foot (under toes):

R plantar distal foot (under 5th digit):

R plantar distal foot (under great toe):

R plantar lateral mid-foot:

R plantar medial mid-foot:

L plantar distal mid-foot (under toes):

L plantar distal foot (under 5th digit):

L plantar distal foot (under great toe):

L plantar lateral mid-foot:

L plantar medial mid-foot:

Skin Condition - Measure and label the patient's skin condition

Callus (Location):

Measurements (1):

Pre-Ulcerative Lesion (Location):

Measurement (2):

Ulcer (Location):

Measurement (3):

Redness (Location):

Measurement (4):

Swelling (Location):

Measurement (5):

Dryness (Location):

Measurement (6):

Maceration (Location):

Measurement (7):

Fissure (Location):

Measurement (8):

Warmth (Location):

Measurement (9):

Does the patient use/wear appropriate shoes and socks?:

Has the patient had prior foot care education?:

How frequently does patient check their own feet?:

Does patient use lotion to moisturize feet?:

If patient falls in high risk category, RN must notify PMP, may need order for SN weekly foot checks

Low Risk (All of the following):

High Risk (One or more of the following):

Physician Notified:

New/Change in order:

Follow-Up Needed:

Date:

Comments:

Gastrointestinal/Genitourinary/Reproductive Assessment

GASTROINTESTINAL ASSESSMENT

GI WNL?:

Nausea and vomiting:

Bowel Issues:

Bowel Incontinence Frequency:

Abdominal:

Hepatic:

Bowel Sounds:

Date of last bowel movement:

Stool Color/Consistency:

Stool characteristics:

Ostomy?:

Ostomy:
Ostomy Bag Change:
Stoma Site Condition:
Appliance/Device Numbers:

Stoma:
Surrounding Skin:
Gastrointestinal - Documentation/Teaching:

GENTOURINARY

WNL:
Urinary Issues:
Other Urinary :
When Does Urinary Incontinence Occur?:
Urinary Signs/Symptoms UTI:
Other S/S of UTI:
Urine Color:
Catheter:
Changed/Placed/Performed by:
Date of insertion:
Catheter Size:
Frequency/How often changed:
Urostomy:
Appliance/Device numbers:
Genitourinary- Documentation/Teaching:

DIALYSIS

*Dialysis:
Dialysis Days:
Dialysis Access Location:
Thrill/Bruit:
Other:
S/Sx Infection:
Dressing:
Dressing Changed as follows:
Dialysis Center Location:
Telephone:
Dialysis -Documentation/Teaching:

REPRODUCTIVE ASSESSMENT

Male:
Female:
Reproductive -Documentation/Teaching:

GG- FUNCTIONAL ABILITIES & GOALS

GG0100. Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

CODING:

3. Independent - Patient completed all the activities by themselves, with or without an assistive device, with no assistance from a helper.

2. Needed Some Help - Patient needed partial assistance from another person to complete any activities.

1. Dependent - A helper completed all the activities for the patient.

8. Unknown

9. Not Applicable

0. Not Assessed

↓ **Enter Codes in Box**

☐

A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, and eating prior to the current illness, exacerbation, or injury.

☐

B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.

☐

C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.

☐

D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury

GG0110. Prior Device Use. Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury.

↓ **Check all that apply**

☐ **A. Manual wheelchair**

☐ **A. Not assessed**

☐ **B. Motorized wheelchair and/or scooter**

☐ **B. Not assessed**

☐ **C. Mechanical lift**

☐ **C. Not assessed**

☐ **D. Walker**

☐ **D. Not assessed**

☐ **E. Orthotics/Prosthetics**

☐ **E. Not assessed**

☐ **Z. None of the above**

☐ **Z. Not assessed**

GG0130. Self-Care	
Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason. Code the patient's discharge goal(s) using the 6-point scale. Use of codes 07, 09, 10 or 88 is permissible to code discharge goal(s).	
Coding: Safety and Quality of Performance - If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided. <i>Activities may be completed with or without assistive devices.</i> 06. Independent - Patient completes the activity by themselves with no assistance from a helper. 05. Setup or clean-up assistance - Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity. 04. Supervision or touching assistance - Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently. 03. Partial/moderate assistance - Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort. 02. Substantial/maximal assistance - Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort. 01. Dependent - Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity. If activity was not attempted, code reason: 07. Patient refused 09. Not applicable - Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury. 10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints) 88. Not attempted due to medical conditions or safety concerns 99. Not assessed	
1. SOC/ROC Performance ↓ Enter Codes in Boxes ↓	
☐	A. Eating: The ability to use suitable utensils to bring food and/or liquid to the mouth and swallow food and/or liquid once the meal is placed before the patient.
☐	B. Oral Hygiene: The ability to use suitable items to clean teeth. Dentures (if applicable): The ability to insert and replace dentures from and to the mouth, and manage equipment for soaking and rinsing them.
☐	C. Toileting Hygiene: The ability to maintain perineal hygiene, adjust clothes before and after voiding or having a bowel movement. If managing an ostomy, include wiping the opening but not managing equipment.
☐	E. Shower/bathe self: The ability to bathe self, including washing, rinsing, and drying self (excludes washing of back and hair). Does not include transferring in/out of tub/shower
☐	F. Upper body dressing: The ability to dress and undress above the waist; including fasteners, if applicable.
☐	G. Lower body dressing: The ability to dress and undress below the waist, including fasteners; does not include footwear.
☐	H. Putting on/taking off footwear: The ability to put on and take off socks and shoes or other footwear that is appropriate for safe mobility; including fasteners, if applicable.

GG0170. Mobility	
Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason. Code the patient's discharge goal(s) using the 6-point scale. Use of codes 07, 09, 10 or 88 is permissible to code discharge goal(s).	
Coding: Safety and Quality of Performance - If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided. <i>Activities may be completed with or without assistive devices.</i> 06. Independent - Patient completes the activity by themselves with no assistance from a helper. 05. Setup or clean-up assistance - Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity. 04. Supervision or touching assistance - Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently. 03. Partial/moderate assistance - Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort. 02. Substantial/maximal assistance - Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort. 01. Dependent - Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity. If activity was not attempted, code reason: 07. Patient refused 09. Not applicable - Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury. 10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints) 88. Not attempted due to medical conditions or safety concerns 99. Not assessed	
1. SOC/ROC Performance ↓ Enter Codes in Boxes ↓	
☐	A. Roll left and right: The ability to roll from lying on back to left and right side, and return

☐	to lying on back on the bed.	
☐	B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.	
☐	C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with no back support.	
☐	D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.	
☐	E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).	
☐	F. Toilet transfer: The ability to get on and off a toilet or commode.	
☐	G. Car Transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.	
☐	I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space.	
☐	J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.	
☐	K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.	
☐	L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.	
☐	M. 1 step (curb): The ability to go up and down a curb or up and down one step.	
☐	N. 4 steps: The ability to go up and down four steps with or without a rail.	
☐	O. 12 steps: The ability to go up and down 12 steps with or without a rail.	
☐	P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.	
		Q. Does patient use wheelchair and/or scooter? ☐ 0. No ☐ 1. Yes ☐ 9. Not assessed
☐	R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.	
		RR1. Indicate the type of wheelchair or scooter used. ☐ 1. Manual ☐ 2. Motorized ☐ 9. Not assessed
☐	S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.	
		SS1. Indicate the type of wheelchair or scooter used. ☐ 1. Manual ☐ 2. Motorized ☐ 9. Not assessed

*****	Musculoskeletal Assessment	*****
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WNL:

Recent falls:

Mobility:

Pain or cramps:

Location:

Redness, Warmth, Swelling:

Limited ROM:

Amputation:

Prosthesis/Appliance:

Musculoskeletal -Documentation/Teaching:

Loc 18.A - Functional Limitations

Functional Limitations:

Able to complete the following IADLs:

Other:

Describe:

Loc 18.B - Activities Permitted

Activities Permitted:

Other:

MAHC 10-Fall Risk Assessment Tool

**ONE POINT ASSESSED FOR EACH CORE
ELEMENT "YES"**

Age 65+:

Diagnosis (3 or more co-existing):

Includes only documented medical diagnosis

Prior history of falls within 3 months:

**An unintentional change in position resulting in
coming to rest on the ground or at a lower level**

Incontinence:

**Inability to make it to the bathroom or commode
in timely manner**

Includes frequency, urgency and/or nocturia

Visual Impairment:

**Includes but not limited to, macular
degeneration, diabetic retinopathies, visual field
loss,**

**age related changes; decline in visual acuity,
accommodation, glare tolerance, depth
perception,**

**and night vision or not wearing prescribed
glasses or having the correct prescription**

Impaired functional mobility:

**May include patients who need help with IADLS
or ADLS or have gait or transfer problems,**

**arthritis, pain, fear of falling, foot problems,
impaired sensation, impaired coordination or**

improper use of assistive devices

Environmental hazards :

**May include but not limited to, poor illumination,
equipment tubing, inappropriate footwear,**

**pets, hard to reach items, floor surfaces that are
uneven or cluttered, or outdoor entry and exits.**

Poly Pharmacy (4 or more prescriptions-any type):

All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall

risk include but not limited to, sedatives, anti-depressants, tranquilizers, narcotics,

antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and

hypoglycemic drugs

Pain affecting level of function:

Pain often affects an individual's desire or ability to move or pain can be a factor in

depression or compliance with safety recommendations

Cognitive impairment:

Could include patients with dementia, Alzheimer's or stroke patients or patients who are

confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits.

Consider patients ability to adhere to the plan of care

Total Score:

A score of 4 or more is considered at risk for falling

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Education provided:

Education provided to :

Timed up and go findings (seconds):

<10 sec = normal <14 sec = not a fall risk

>14 sec = increased risk for falls

For all above identified risks the patient and

caregiver will be educated and receive appropriate

PT/OT/ST/MSW/AIDE referrals.

Fall Risk - Supporting Documentation:

Vulnerability and Safety Assessment

Mental

Patient confused/disoriented?:

Plan of Action:

Notification:

Limited decision-making present?:

Plan of Action:

Notification:

Patient at risk for mental or financial exploitation?:

Plan of Action:

Notification:

Patient incompetent/uncooperative with plan of care?:

Plan of Action:

Notification :

Patient misuse/abuse of substances?:

Plan of Action:

Notification:

Patient presents risk of abusing others?:

Plan of Action:

Notification:

Physical

Patient able to ambulate safely?:

Plan of Action:

Notification:

Sensory deficit present that affects safety?:

Plan of Action:

Notification:

Functional limitation present that affect safety?:

Plan of Action:

Notification:

Patient demonstrates self-abusive behavior?:

Plan of Action:

Notification:

Patient is at risk of abuse from others?:

Plan of Action:

Notification:

ENVIRONMENTAL

Inadequate finances?:

Plan of Action:

Notification:

Inadequate housing? :

Plan of Action:

Notification:

Home environment is not safe and secure? :

Plan of Action:

Notification:

Unsanitary home environment?:

Plan of Action:

Notification:

Inadequate or unavailable access to food and medical
care? :

Plan of Action:

Notification:

Patient is dependent on caregiver? :

Plan of Action:

Notification:

Safety equipment available:

Plan of Action:

Notification:

Updated February 19, 2018

**Approved by Governing Body and Medical
Director**

Safety Hazards in the Home

HOME ENVIRONMENT

Home Environment - Safety Hazards Present?:

Identified Safety Hazards:

Safety/Needs Documentation:

Fire Risk Assessment

Oxygen in use in home?:

"No Smoking" sign posted:

Functioning smoke detector:

Intact Electrical Cords:

Oxygen stored in well vented area:

No smoking in home:

No open flames in home (candles, lighters):

No Petroleum products near flow of O2:

Fire Risk- Additional Documentation:

Loc 15 - Safety Measures

Safety Measures:

Other:

Loc 14 - Supplies/DME

Supplies/DME:

Other:

Is patient receiving supplies from any other provider?:

Name of Provider:

Address:

Phone Number:

List of items received from other provider:

H - BLADDER AND BOWEL**(M1600)** Has this patient been treated for a **Urinary Tract Infection** in the past 14 days?☐ 0 - No☐ 1 - Yes☐ NA - Patient on prophylactic treatment☐ UK - Unknown**(M1610) Urinary Incontinence or Urinary Catheter Presence:**☐ 0 - No incontinence or catheter (includes anuria or ostomy for urinary drainage)☐ 1 - Patient is incontinent☐ 2 - Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)**(M1620) Bowel Incontinence Frequency:**☐ 0 - Very rarely or never has bowel incontinence☐ 1 - Less than once weekly☐ 2 - One to three times weekly☐ 3 - Four to six times weekly☐ 4 - On a daily basis☐ 5 - More often than once daily

☐ NA - Patient has ostomy for bowel elimination

☐ UK - Unknown

(M1630) Ostomy for Bowel Elimination: Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

☐ 0 - Patient does not have an ostomy for bowel elimination.

☐ 1 - Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.

☐ 2 - The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

I - ACTIVE DIAGNOSES

(M1021) Primary Diagnosis & (M1023) Other Diagnoses	
Column 1	Column 2
Diagnoses (Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided.)	ICD-10-CM and symptom control rating for each condition. Note that the sequencing of these ratings may not match the sequencing of the diagnoses
Description	ICD-10-CM/Symptom Control Rating
(M1021) Primary Diagnosis a.	V, W, X, Y codes NOT allowed a. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
(M1023) Other Diagnoses b.	All ICD-10-C M codes allowed b. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
c.	c. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
d.	d. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
e.	e. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
f.	f. ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
Additional Diagnosis Codes	
g.	h.
i.	j.
k.	l.
m.	n.
o.	p.
q.	r.
s.	t.
u.	v.
w.	x.
y.	
Procedure Codes	

(M1028) Active Diagnoses - Comorbidities and Co-existing Conditions - Check all that apply

☐ 1 - Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)

☐ - Not Assessed

☐ 2 - Diabetes Mellitus (DM)

☐ - Not Assessed

☐ 3 - None of the above

J - HEALTH CONDITIONS

(M1033) **Risk for Hospitalization:** Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ 1 - History of falls (2 or more falls - or any fall with an injury - in the past 12 months)
- ☐ 2 - Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- ☐ 3 - Multiple hospitalizations (2 or more) in the past 6 months
- ☐ 4 - Multiple emergency department visits (2 or more) in the past 6 months
- ☐ 5 - Decline in mental, emotional, or behavioral status in the past 3 months
- ☐ 6 - Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- ☐ 7 - Currently taking 5 or more medications
- ☐ 8 - Currently reports exhaustion
- ☐ 9 - Other risk(s) not listed in 1 - 8
- ☐ 10 - None of the above

(J0510) **Pain effect on sleep**

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night?"

- ☐ 0 - Does not apply - I have not had any pain or hurting in the past 5 days → Skip to M1400, Short of Breath at SOC/ROC; Skip to J1800, Any Falls Since SOC/ROC at DC
- ☐ 1 - Rarely or not at all
- ☐ 2 - Occasionally
- ☐ 3 - Frequently
- ☐ 4 - Almost constantly
- ☐ 8 - Unable to answer

(J0520) **Pain Interference with Therapy Activities**

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

- ☐ 0 - Does not apply - I have not received rehabilitation therapy in the past 5 days
- ☐ 1 - Rarely or not at all
- ☐ 2 - Occasionally
- ☐ 3 - Frequently
- ☐ 4 - Almost constantly
- ☐ 8 - Unable to answer

(J0530) **Pain inference with day-to-day activities**

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy sessions) because of pain?"

- ☐ 1 - Rarely or not at all
- ☐ 2 - Occasionally
- ☐ 3 - Frequently
- ☐ 4 - Almost constantly
- ☐ 8 - Unable to answer

J - HEALTH CONDITIONS (RESPIRATORY)

(M1400) When is the patient dyspneic or noticeably **Short of Breath?**

- ☐ 0 - Patient is not short of breath
- ☐ 1 - When walking more than 20 feet, climbing stairs
- ☐ 2 - With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ☐ 3 - With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ☐ 4 - At rest (during day or night)

Influenza

Is this visit being done during Influenza Season of
October 1st through March 31st?:

Inactivated Influenza Risk Assessment

Patient is 50 years of age or older:

Patient is 18-64 years of age and has any of the following high-risk conditions:

Influenza risk assessment indicates that patient is at risk of getting influenza:

Influenza contradiction assessment

Influenza vaccine not indicated if any apply:

Influenza vaccine contradicted for patient:

Medications Assessment

4-M Is patient taking age friendly medications that does not interfere with what matters most, mentation and mobility?:

Patient/Family/CG can safely administer medications:

Drug Regimen Review - List problems identified:

Medication Follow-Up:

Name of Physician called:

Date/Time MD ackn.receipt of info and/or orders:

Pt/Caregiver High Risk Drug Education:

List high risk med(s) identified:

Medications Reviewed for:

Physician notified for any medication discrepancies, side effects, problems, reactions, duplicate drug therapy.:

Comments:

Education provided:

Prior Medication Management:

Medication-Documentation/Teaching:

Intravenous Therapy / Injection

IV THERAPY:

Intravenous Line - Type:

Location:

Date Inserted:

Date IV Dressing last changed:

Flushed with:

IV Therapy- Documentation/Teaching:

INJECTION ADMINISTERED:

Other:

Medication Administered:

Dosage:

Location Administered:

Injection Documentation:

PRECAUTIONS:

Central Line Care

Catheter Type:

Other, type:

Line Length:
Location:
Number of Lumens:
Date Inserted:
Interventions:
Huber Needle Size:
Techniques Used:
Site Assessment:
Date Dressing Changed:
Date Next Dressing Change:
Supplies Used:
(details):

***** Lab *****

LABORATORY DATA

Lab Obtained:
Lab Delivered to:
PRECAUTIONS:
V/P performed to:
Other:
x (# of tries):
Pt tolerated :
Site Dressed:
Lab Note:

K - SWALLOWING/NUTRITIONAL STATUS

(M1060) Height and Weight - While measuring, if the number is X.1 - X.4 round down; X.5 or greater round up

☐ - Not Assessed

Inches

☐ a.Height (in inches). Record most recent height measure since the most recent SOC/ROC

☐ - Not Assessed

Pounds

☐ b.Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.)

(K0520) Nutritional Approaches

Check all of the nutritional approaches that apply on admission

☐ A - Parenteral/IV feeding

☐ B - Feeding tube (e.g., nasogastric or abdominal (PEG))

☐ C - Mechanically altered diet - require change in texture of food or liquids (e.g., pureed food, thickened liquids)

☐ D - Therapeutic diet (e.g., low salt, diabetic, low cholesterol)

☐ Z - None of the above

☐ Not Assessed

(M1870) Feeding or Eating: Current ability to feed self meals and snacks safely. Note: This refers only to the process of eating, chewing, and swallowing, not preparing the food to be eaten.

☐ 0 - Able to independently feed self.

☐ 1 - Able to feed self independently but requires:

(a) meal set-up; OR

(b) intermittent assistance or supervision from another person; OR

(c) a liquid, pureed or ground meat diet.

- 2 - Unable to feed self and must be assisted or supervised throughout the meal/snack.
- 3 - Able to take in nutrients orally and receives supplemental nutrients through a nasogastric tube or gastrostomy.
- 4 - Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- 5 - Unable to take in nutrients orally or by tube feeding.

*******Home Health Disaster Level*******

Disaster Risk Level:

Why is this risk level chosen:

Individual emergency plan to include but not limited to evacuation destination,

alternate destination, transportation plan, high risk factors, and communication plan

Individual emergency plan details:

M - SKIN CONDITION

(M1306) Does this patient have at least one **Unhealed Pressure Ulcer/Injury at Stage 2 or Higher** or designated as Unstageable? (Excludes Stage 1 pressure ulcers and healed Stage 2 pressure ulcers/injuries)

☐ 0 - No

☐ 1 - Yes

(M1311)

Current Number of Unhealed Pressure Ulcers at Each Stage:	Enter Number
A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with red pink wound bed, without slough. May also present as an intact or open/ruptured blister. Number of Stage 2 pressure ulcers	
B1. Stage 3: Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. Number of Stage 3 pressure ulcers	
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. Number of Stage 4 pressure ulcers	
D1. Unstageable: Non-removable dressing: Known but not stageable due to non-removable dressing/device Number of unstageable pressure ulcers due to non-removable dressing/device	
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar	
F1. Unstageable: Deep tissue injury: Suspected deep tissue injury in evolution Number of unstageable pressure ulcers with suspected deep tissue injury in evolution	

(M1322) **Current Number of Stage 1 Pressure Injuries:** Intact skin with non-blanchable redness of a localized area usually over a bony prominence. The area may be painful, firm, soft, warmer, or cooler as compared to adjacent tissue. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.

☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4 or more

(M1324) **Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:** (Excludes pressure ulcer that cannot be staged due to a nonremovable dressing/device, coverage of wound bed by slough and/or eschar, or suspected deep tissue injury.)

☐ 1 - Stage 1

☐ 2 - Stage 2

☐ 3 - Stage 3

☐ 4 - Stage 4

☐ NA - Patient has no pressure ulcers or no stageable pressure ulcers

(M1330) Does this patient have a **Stasis Ulcer**?

☐ 0 - No

☐ 1 - Yes, patient has BOTH observable and unobservable stasis ulcers

☐ 2 - Yes, patient has observable stasis ulcers ONLY

☐ 3 - Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non-removable dressing/device)

(M1332) **Current Number of Stasis Ulcer(s) that are Observable:**

☐ 1 - One

☐ 2 - Two

☐ 3 - Three

☐ 4 - Four or more

(M1334) **Status of Most Problematic Stasis Ulcer that is Observable:**

☐ 1 - Fully granulating

☐ 2 - Early/partial granulation

☐ 3 - Not healing

(M1340) Does this patient have a **Surgical Wound**?

☐ 0 - No

☐ 1 - Yes, patient has at least one observable surgical wound

☐ 2 - Surgical wound known but not observable due to non-removable dressing/device

(M1342) **Status of Most Problematic Surgical Wound that is Observable:**

☐ 0 - Newly epithelialized

☐ 1 - Fully granulating

☐ 2 - Early/partial granulation

☐ 3 - Not healing

*****	Loc - Care Coordination/POC Collaboration	*****
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Attending/Certifying Physician notified and VO for
POC received:
Collaborated with additional Physicians involved in
patient's care :
Care Coordination and Plan of Care Collaboration
with:
Names of physicians that collaborated in POC:
The following disciplines may be indicated based on
assessment:
Additional discipline(s) indicated due to:
If Pt refused any of above, specify disc and reason:
Physician contacted and additional orders received for
above indicated disciplines?:
OTHER SERVICE PROVIDERS:
Other service providers involved or needed?:
Service Providers Comments/Needs:
Additional Details:

*****	Loc - Patient/CG Assessment	*****
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**Emergency Contact/Patient
Representative/Legal Guardian/Responsible
Party/Caregiver Name and Number**

Emergency Contact Name and Number:
Patient Representative Name and Number:
Legal Guardian Name and Number:
Primary Caregiver Name and Number:
Primary Caregiver:
Primary Caregiver Schedules for Care:
Caregiver needs identified:
Describe:
Interventions to assist with needs:
Role changes and family dynamics:
Client's involvement with social and community
resources:
Financial, economic and community resources:

*****	Loc - Patient Stated Goals and Preferences	*****
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4M - How does the patient wish to be addressed:
Patient Strengths, Preferences, Goals
Patient Strengths:
Patient Preferences to include requested schedules:
Patient/Family Goals:

*****	Care Management Information	*****
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OTHER SERVICE PROVIDERS:

Service Providers Comments/Needs:

Other physicians added to profile:

Names/Specialties/Phone Numbers:

**PREVENTATIVE / PERIODIC HEALTH
SCREENING**

IMMUNIZATIONS:

Influenza - date received:

Pneumonia - date received:

Tetanus-Diphtheria - date received:

Tuberculosis Exposure?:

SCREENINGS:

Patient Current?:

SELF-EXAM FREQUENCY:

Breast:

Testicular:

Other:

***** **Loc 20 - Prognosis** *****

Prognosis:

***** **Admission/ROC Summary** *****

Prior Medical/Surgical History/Recent Hospital Stays:

SN/Therapy Narrative Note:

Skilled Intervention/Procedure Provided this visit:

Patient/Caregiver response to above care/teaching:

Plan for Next Visit:

Pt/Cgr participated in the plan of care & is :

aware of treatment options:

Homebound Status:

Additional Homebound Information:

Discharge Plans:

Specify:

MD cntct'd to approve add'l orders not on referral:

Date:

Time:

Spoke with:

***** **Loc 22 Rehab Potential/Discharge Plans - SN** *****

SN Discharge Plans:

Patient Involvement:

Rehab Potential (select):

***** **Loc 22 Rehab Potential/Discharge Plans - HHA** *****

HHA Discharge Plans:

Patient Involvement:

Rehab Potential (select):

(N0415)

High-Risk Drug Classes: Use and Indication

1. Is taking

Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes

- ☐ A. Antipsychotic
- ☐ E. Anticoagulant
- ☐ F. Antibiotic
- ☐ H. Opioid
- ☐ I. Antiplatelet
- ☐ J. Hypoglycemic
- ☐ Z. None of the above
- ☐ Not Assessed

2. Indication noted

If 'IS Taking' is checked, check if there is an indication noted for all medications in the drug class

- ☐ A. Antipsychotic
- ☐ E. Anticoagulant
- ☐ F. Antibiotic
- ☐ H. Opioid
- ☐ I. Antiplatelet
- ☐ J. Hypoglycemic

(M2001) Drug Regimen Review: Did a complete drug regimen review identify potential clinically significant medication issues?

- ☐ 0 - No - No issues found during review
- ☐ 1 - Yes - Issues found during review
- ☐ 9 - NA - Patient is not taking any medications
- ☐ - NA - Not Assessed

(M2003) Medication Follow-up: Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ☐ 0 - No
- ☐ 1 - Yes
- ☐ - NA - Not Assessed

(M2010) Patient/Caregiver High Risk Drug Education: Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ☐ 0 - No
- ☐ 1 - Yes
- ☐ NA - Patient not taking any high risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications

(M2020) Management of Oral Medications: Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. **Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)**

- ☐ 0 - Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.
- ☐ 1 - Able to take medication(s) at the correct times if:
 - (a) individual dosages are prepared in advance by another person; OR
 - (b) another person develops a drug diary or chart.
- ☐ 2 - Able to take medication(s) at the correct times if given reminders by another person at the appropriate times
- ☐ 3 - Unable to take medication unless administered by another person.
- ☐ NA - No oral medications prescribed.

(M2030) Management of Injectable Medications: Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. **Excludes IV medications.**

- ☐ 0 - Able to independently take the correct medication(s) and proper dosage(s) at the correct times.
- ☐ 1 - Able to take injectable medication(s) at the correct times if:
 - (a) individual syringes are prepared in advance by another person; OR
 - (b) another person develops a drug diary or chart.
- ☐ 2 - Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection
- ☐ 3 - Unable to take injectable medication unless administered by another person.
- ☐ NA - No injectable medications prescribed.

O - SPECIAL TREATMENTS, PROCEDURES, AND PROGRAMS

(O0110) Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply on admission.

Cancer Treatments

☐ A1. Chemotherapy

If A1 is selected, select all that apply:

☐ A2. IV

☐ A3. Oral

☐ A10. Other

☐ B1. Radiation

Respiratory Therapies

☐ C1. Oxygen Therapy

If C1 is selected, select all that apply:

☐ C2. Continuous

☐ C3. Intermittent

☐ C4. High-concentration

☐ D1. Suctioning

If D1 is selected, select all that apply:

☐ D2. Scheduled

☐ D3. As Needed

☐ E1. Tracheostomy care

☐ F1. Invasive Mechanical Ventilator (ventilator or respirator)

☐ G1. Non-invasive Mechanical Ventilator

If G1 is selected, select all that apply:

☐ G2. BiPAP

☐ G3. CPAP

Other

☐ H1. IV Medications

If H1 is selected, select all that apply:

☐ H2. Vasoactive medications

☐ H3. Antibiotics

☐ H4. Anticoagulation

☐ H10. Other

☐ I1. Transfusions

☐ J1. Dialysis

If J1 is selected, select all that apply:

☐ J2. Hemodialysis

☐ J3. Peritoneal dialysis

☐ O1. IV Access

If O1 is selected, select all that apply:

☐ O2. Peripheral

☐ O3. Mid-line

☐ O4. Central (e.g., PICC, tunneled, port)

None of the Above

☐ Z1. None of the Above

☐ No Information

I hereby attest that the information that I documented for this patient's visit on this date is true, accurate, and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Electronically signed and dated by:

Signature	Associate Name	User ID	Date	Time	Time Zone